

TASK 2 – ADR REPORTING & DOCUMENTATION

Completed Structured Adverse Drug Reaction (ADR) Report

Task ID: task2

1. PATIENT INFORMATION

Patient Identifier	Patient A (de-identified)
Age	45 years
Gender	Female

2. SUSPECTED MEDICINE

Medicine Name	Amoxicillin
Indication	Not specified in the case
Route	Not documented
Dose / Frequency	Not documented
Start Date	Not documented; therapy had been started 5 days before the report

3. ADVERSE REACTION / EVENT

Reaction / Event	Rash, itching, nausea, and dizziness
Time to Onset	Symptoms began approximately 3 days after starting amoxicillin
Course	Symptoms persisted after onset
Outcome	Not documented in the available case information
Seriousness	Not serious based on the available information; no hospitalization, disability, life-threatening event, or death was reported

4. REPORTER INFORMATION

Reporter	Not provided / simulated training report
Designation	Pharmacovigilance Trainee
Healthcare Institution	Not specified
Contact	Not applicable

5. ADDITIONAL INFORMATION

Concomitant Medication	Paracetamol used as needed (PRN)
Relevant Observation	Paracetamol is considered less likely to be responsible because the symptoms developed after amoxicillin was initiated and the clinical pattern is more consistent with an amoxicillin-related hypersensitivity reaction.
Dechallenge	Not documented
Rechallenge	Not performed / not documented

6. PHARMACOVIGILANCE ASSESSMENT

Suspected Drug	Amoxicillin
Suspected ADR	Rash, itching, nausea, and dizziness
Causality Assessment	Probable
Reason for Assessment	There is a reasonable temporal relationship: symptoms appeared about 3 days after amoxicillin was started. Rash and itching are compatible with a hypersensitivity/allergic reaction to amoxicillin. A competing medicine (paracetamol PRN) is considered less likely based on the available case information.
Expectedness	Rash and hypersensitivity-type reactions are known adverse effects associated with amoxicillin.
Alternative Causes	No specific alternative cause was provided in the case.

7. CONCLUSION

The reported event is considered a suspected adverse drug reaction associated with **amoxicillin**. Based on the available information, the relationship is assessed as **probable**, primarily because of the appropriate temporal association and the compatibility of the reported symptoms with a known hypersensitivity reaction. Further information such as dose, route, exact treatment dates, dechallenge outcome, relevant medical history, and clinical management would strengthen the report.

8. REPORTING CHECKLIST

■	Patient demographics recorded
■	Suspected medicine identified
■	Adverse reactions documented
■	Time to onset documented
■	Concomitant medicine considered
■	Causality assessed
■	Missing information clearly marked as not documented

Note: This document is a simulated pharmacovigilance training report prepared from the case information available for the task. It is not a real patient safety report.